

EATING WELL AS YOU AGE: WHY PROTEIN AND CONSISTENCY MATTER MOST

A conservative, protein-first approach to fueling for strength and independence later in life.

A lot of nutrition advice aimed at older adults swings between two extremes: either it is the same aggressive deficit language written for a 25-year-old trying to get lean, or it is vague reassurance with no real numbers at all. Neither is very useful. The goal here is neither losing weight fast nor "eating big", it is making sure you are eating enough, and enough of the right things, to support the strength and independence that matter most at this stage.

Starting With a Baseline: BMR

Basal metabolic rate (BMR) is simply the energy your body burns at rest, keeping your organs, brain, and basic functions running. It is a useful starting reference point, not a strict rule. The Mifflin-St Jeor equation estimates it from weight, height, and age:

- **Men:** $\text{BMR} = (10 \times \text{weight in kg}) + (6.25 \times \text{height in cm}) - (5 \times \text{age}) + 5$
- **Women:** $\text{BMR} = (10 \times \text{weight in kg}) + (6.25 \times \text{height in cm}) - (5 \times \text{age}) - 161$

Take a 63-year-old woman starting a functional-strength and mobility program, 70 kg (about 154 lbs), 163 cm tall (about 5'4"). Her BMR: $(10 \times 70) + (6.25 \times 163) - (5 \times 63) - 161 = 700 + 1,018.75 - 315 - 161 = 1,243$ calories.

BMR to TDEE: A Gentle, Realistic Picture

Multiply BMR by an activity multiplier to estimate total daily energy needs (TDEE): sedentary $\times 1.2$, lightly active $\times 1.375$, moderately active $\times 1.55$, very active $\times 1.725$. A program with three functional-strength sessions a week, daily mobility work, and regular walking lands at lightly active. $1,243 \times 1.375 = 1,709$ calories, a reasonable estimate of what it takes to maintain her current weight.

Why This Isn't About a Deficit or a Surplus

For most other goals, the next step is adjusting that number up or down with some intention behind it. Here, the more important message is different: the biggest, most common mistake in this population isn't eating too much, it's eating too little, often without realizing it, as appetite naturally decreases with age even while the body's need for protein and adequate energy stays real. Rather than a deficit or surplus, the priority is landing close to that maintenance number and staying consistent, with adjustments made carefully and gradually, ideally alongside a conversation with a doctor or dietitian rather than a generic

percentage cut.

The One Number That Matters Most: Protein

If there's a single lever worth focusing on here, it's protein. Adequate protein intake, spread across meals through the day rather than saved for dinner, supports muscle maintenance, which is directly tied to strength, balance, and staying independent with everyday tasks like stairs and carrying groceries. A simple starting target: a palm-sized portion of protein at most meals, three to four times a day, alongside vegetables for fiber and micronutrients and enough overall food to avoid the under-eating pattern that's so common in this age group.

A Brief Note on Supplements

If getting enough protein through food alone is a genuine struggle, a whey option like **XS™ Whey Protein** can be a simple, gentle way to close that gap. Some clients also look into creatine as a general strength-support option, and **XS™ Creatine+** is available for anyone who wants to explore that with their own research and their doctor's input, but for this population, food, consistency, and protein are the real priorities, not supplements. Details on both are available through [the Echelon Goods shop](#).

What This Means for You

- Your calorie needs still start with BMR and an activity factor, the same math as any other goal, just applied gently here.
- Under-eating, not overeating, is the more common and more concerning pattern in this population, staying close to maintenance is usually the right call.
- Protein, spread across the day, is the single biggest nutrition lever for maintaining strength and independence as you age.
- Because age often comes with medical conditions or medications that affect nutrition needs, checking with your doctor before changing your eating pattern matters more here than in almost any other goal.

This article is educational and general in nature, not individualized medical or nutrition advice. Because medical conditions and medications become more common with age, please talk to your doctor or a registered dietitian before making changes to your eating pattern, especially around protein intake or any supplement use.